

1 **HOUSE OF REPRESENTATIVES - FLOOR VERSION**

2 STATE OF OKLAHOMA

3 2nd Session of the 60th Legislature (2026)

4 COMMITTEE SUBSTITUTE
5 FOR
6 HOUSE BILL NO. 4430

By: Hilbert

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9 COMMITTEE SUBSTITUTE

10 An Act relating to medical malpractice; amending 59
11 O.S. 2021, Section 519.6, as amended by Section 5,
12 Chapter 343, O.S.L. 2025 (59 O.S. Supp. 2025, Section
13 519.6), which relates to license required,
14 supervision, and practice agreements within the
15 Physician Assistant Act; amending Section 2, Chapter
16 340, O.S.L. 2025 (59 O.S. Supp. 2025, Section
17 567.5b), which relates to Advanced Practice
18 Registered Nurse malpractice insurance requirements;
19 providing for compliance; and providing an effective
20 date.

21 BE IT ENACTED BY THE PEOPLE OF THE STATE OF OKLAHOMA:

22 SECTION 1. AMENDATORY 59 O.S. 2021, Section 519.6, as
23 amended by Section 5, Chapter 343, O.S.L. 2025 (59 O.S. Supp. 2025,
24 Section 519.6), is amended to read as follows:

1 Section 519.6. A. No health care services may be performed by
2 a physician assistant unless a current license is on file with and
3 approved by the State Board of Medical Licensure and Supervision.

4 B. A physician assistant with six thousand two hundred forty
5 (6,240) or more hours of postgraduate clinical practice experience
6 who has reported those hours to the Board shall not be required to
7 practice under the supervision of a delegating physician.

8 1. A physician assistant may report the completion of
9 postgraduate clinical practice experience to the Board at any time
10 after completion of at least six thousand two hundred forty (6,240)
11 such hours.

12 2. Hours earned prior to the effective date of this act shall
13 be counted towards the six thousand two hundred forty (6,240) hours.

14 3. The Board shall maintain, make available, and keep updated,
15 on the Internet website of the Board, a list of physician assistants
16 who have reported completion of six thousand two hundred forty
17 (6,240) or more postgraduate clinical practice experience hours.

18 4. The Board shall prescribe a form for reporting postgraduate
19 clinical practice experience by a physician assistant. The Board
20 shall make available and keep updated on the Internet website of the
21 Board the prescribed form. This reporting form may be filed
22 electronically. The Board shall not charge a fee for reporting
23 hours or filing of the prescribed form.

1 5. Nothing in this subsection shall prohibit a physician
2 assistant from maintaining a practice agreement; however, such an
3 agreement is not required for a physician assistant with the
4 reported six thousand two hundred forty (6,240) hours of
5 postgraduate clinical practice experience, provided any practice
6 agreements are subject to the requirements of paragraphs 1, 2, 3,
7 and 4 of subsection C of this section.

8 6. Nothing in this subsection shall restrict the ability of the
9 Board to require supervision as a part of disciplinary action
10 against the license of a physician assistant.

11 C. A physician assistant with less than six thousand two
12 hundred forty (6,240) hours of postgraduate clinical practice
13 experience or who has completed six thousand two hundred forty
14 (6,240) hours but has not reported those hours to the Board shall
15 practice under the supervision of a delegating physician with the
16 following requirements:

17 1. All practice agreements and any amendments shall be filed
18 with the State Board of Medical Licensure and Supervision within ten
19 (10) business days of being executed. Practice agreements may be
20 filed electronically. The State Board of Medical Licensure and
21 Supervision shall not charge a fee for filing practice agreements or
22 amendments to practice agreements;

23 2. A physician assistant may have practice agreements with
24 multiple allopathic or osteopathic physicians. Each physician shall

1 be in good standing with the State Board of Medical Licensure and
2 Supervision or the State Board of Osteopathic Examiners;

3 3. The delegating physician need not be physically present nor
4 be specifically consulted before each delegated patient care service
5 is performed by a physician assistant, so long as the delegating
6 physician and physician assistant are or can be easily in contact
7 with one another by means of telecommunication. The delegating
8 physician shall provide appropriate methods of participating in
9 health care services provided by the physician assistant including:

- 10 a. being responsible for the formulation or approval of
11 all orders and protocols, whether standing orders,
12 direct orders or any other orders or protocols, which
13 direct the delivery of health care services provided
14 by a physician assistant, and periodically reviewing
15 such orders and protocols,
- 16 b. regularly reviewing the health care services provided
17 by the physician assistant and any problems or
18 complications encountered,
- 19 c. being available physically or through telemedicine or
20 direct telecommunications for consultation, assistance
21 with medical emergencies or patient referral,
- 22 d. reviewing a sample of outpatient medical records.
23 Such reviews shall take place at a site agreed upon
24 between the delegating physician and physician

1 assistant in the practice agreement which may also
2 occur using electronic or virtual conferencing, and
3 e. that it remains clear that the physician assistant is
4 an agent of the delegating physician; but, in no event
5 shall the delegating physician be an employee of the
6 physician assistant;

7 4. In patients with newly diagnosed complex illnesses, the
8 physician assistant shall contact the delegating physician within
9 forty-eight (48) hours of the physician assistant's initial
10 examination or treatment and schedule the patient for appropriate
11 evaluation by the delegating physician as directed by the physician.
12 The delegating physician shall determine which conditions qualify as
13 complex illnesses based on the clinical setting and the skill and
14 experience of the physician assistant.

15 D. A physician assistant not practicing under a practice
16 agreement may prescribe written and oral prescriptions and orders.
17 The physician assistant not practicing under a practice agreement
18 may prescribe medical supplies, services, and drugs, including
19 controlled medications in Schedules III through V pursuant to
20 Section 2-312 of Title 63 of the Oklahoma Statutes. Physician
21 assistants not practicing under a practice agreement may not
22 dispense drugs, but may request, receive, and sign for professional
23 samples and may distribute professional samples to patients.

1 E. A physician assistant practicing under a practice agreement
2 may prescribe written and oral prescriptions and orders. The
3 physician assistant practicing under a practice agreement may
4 prescribe medical supplies, services, and drugs, including
5 controlled medications in Schedules II through V pursuant to Section
6 2-312 of Title 63 of the Oklahoma Statutes, written and oral
7 prescriptions and orders only as delegated by the delegating
8 physician, and prescriptions and orders for Schedule II drugs
9 written by such physician assistant shall be included on a written
10 protocol determined by the delegating physician. Physician
11 assistants practicing under a practice agreement may not dispense
12 drugs, but may request, receive, and sign for professional samples
13 and may distribute professional samples to patients. Provided that
14 a physician assistant practicing under a practice agreement may not
15 prescribe any controlled medications in a Schedule that the
16 delegating physician is not registered to prescribe.

17 F. Each physician assistant licensed under the Physician
18 Assistant Act shall keep his or her license available for inspection
19 at the primary place of business and shall, when engaged in
20 professional activities, identify himself or herself as a physician
21 assistant.

22 G. A physician assistant shall be bound by the provisions
23 contained in Sections 725.1 through 725.5 of this title.
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1 H. 1. A physician assistant not practicing under a practice
2 agreement, or the employer of such physician assistant on his or her
3 behalf, shall carry malpractice insurance or demonstrate proof of
4 financial responsibility in a minimum amount of One Million Dollars
5 (\$1,000,000.00) per occurrence and Three Million Dollars
6 (\$3,000,000.00) in the aggregate per year. This requirement shall
7 not apply to a physician assistant practicing under a practice
8 agreement.

9 2. A physician assistant who is employed by or under contract
10 with a federal agency that carries malpractice insurance in any
11 amount on behalf of the physician assistant shall be deemed in
12 compliance with paragraph 1 of this subsection when practicing under
13 such federal employment or contract. However, to the extent the
14 physician assistant practices outside of such federal employment or
15 contract, the physician assistant, or his or her employer, shall
16 comply with paragraph 1 of this subsection.

17 3. A physician assistant who is employed by a state agency or
18 facility that is covered by or subject to The Governmental Tort
19 Claims Act, Section 151 et seq. of Title 51 of the Oklahoma
20 Statutes, shall be deemed in compliance with paragraph 1 of this
21 subsection when practicing under such state employment. However, to
22 the extent the physician assistant practices outside of such state
23 employment, the physician assistant shall comply with paragraph 1 of
24 this subsection.

SECTION 2. AMENDATORY Section 2, Chapter 340, O.S.L.

2025 (59 O.S. Supp. 2025, Section 567.5b), is amended to read as follows:

Section 567.5b. A. An Advanced Practice Registered Nurse, or the employer of the Advanced Practice Registered Nurse on his or her behalf, shall carry malpractice insurance or demonstrate proof of financial responsibility in a minimum amount of One Million Dollars (\$1,000,000.00) per occurrence and Three Million Dollars (\$3,000,000.00) in the aggregate per year. This requirement shall apply only to the Advanced Practice Registered Nurse and shall not be construed as to require the Advanced Practice Registered Nurse to provide malpractice insurance coverage to any supervising physician.

B. An Advanced Practice Registered Nurse who is employed by or under contract with a federal agency that carries malpractice insurance in any amount on behalf of the Advanced Practice Registered Nurse shall be deemed in compliance with subsection A of this section when practicing under such federal employment or contract. However, to the extent the Advanced Practice Registered Nurse practices outside of such federal employment or contract, the Advanced Practice Registered Nurse, or his or her employer, shall comply with subsection A of this section.

C. An Advanced Practice Registered Nurse who is employed by a state agency or facility that is covered by or subject to The Governmental Tort Claims Act, Section 151 et seq. of Title 51 of the

1 Oklahoma Statutes, shall be deemed in compliance with subsection A
2 of this section when practicing under such state employment.
3 However, to the extent the Advanced Practice Registered Nurse
4 practices outside of such state employment, the Advance Practice
5 Registered Nurse shall comply with subsection A of this section.

6 SECTION 3. This act shall become effective November 1, 2026.
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8 COMMITTEE REPORT BY: COMMITTEE ON HEALTH AND HUMAN SERVICES
9 OVERSIGHT, dated 03/05/2026 - DO PASS, As Amended.
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